freecare

Should your Maryland hospital bill be free?

Find out in a minute. Then let the agent do the paperwork.

Ayush Kadigari, University of Maryland '27, Information Science and Economics

Pitch Dingman Competition, Fall 2026 Idea Track

PROBLEM

Maryland law makes hospital care free below 200% of the poverty line. Most families who qualify still pay.

- 29% of patients with bills they cannot afford ever learn about, apply for and receive hospital financial assistance (Dollar For)
- $14 billion a year in assistance hospitals fail to provide is booked as bad debt and sent to collections (Dollar For)
- 1.24 million Marylanders live under 200% of the federal poverty line (KFF, 2024); Maryland hospitals carried $1.12 billion of uncompensated care in FY2025 (HSCRC)
- The bill arrives. Nobody mentions the policy. The 240-day window runs out. This started at home when my mom was diagnosed with cancer.

CUSTOMER

The family that opens a hospital bill and assumes the number is final.

- Maryland patients at or under 300% of the poverty line, insured or not: a family of four up to $66,000 owes $0; up to $99,000 gets at least 60% off
- Anyone on SNAP, WIC, energy assistance or free school meals: automatically free, and most do not know it. Immigration status does not matter in Maryland
- First users: families and students around College Park and Prince George's County, reached through clinics, legal aid and the UMD Health Center
- Discovery so far: the statute and regulation encoded and tested; hospital policies read (UMMS: a 14-page PDF); Dollar For's survey data. Next 30 days: 20 patient and 5 counselor interviews

SOLUTION

Answer four questions, get the law's answer in a minute, then let the agent do the paperwork.

- 1. Check (built): household size, income, residency, first-bill date. Free, 75% off, 60% off or a hardship tier, days left, what to bring. Runs in the browser; nothing is sent anywhere. 53 automated tests
- 2. Read the bill (building): snap a photo; the agent pulls the hospital, dates and balance, picks that hospital's policy, starts the 240-day clock
- 3. File (building): fills Maryland's uniform financial assistance application, lists documents, reminds the family until the hospital answers
- Why it endures: Maryland's rules are specific, the form is statewide, and the bill never leaves the patient's phone

MARKET AND WHY NOW

Since December 11, 2025, Maryland hospitals must determine eligibility before they file to collect. Nobody has built the patient's side.

- HSCRC rules: one statewide application, automatic eligibility for SNAP and WIC families, assistance regardless of status; hospitals refiled policies by April 30, 2026
- Maryland first, then every state with a free-care law: Washington, Oregon, California, Illinois, New Jersey set free-care lines at 200% of the poverty line or higher
- Each new state is a rules file and a form, not a new product

COMPETITION

Today a family finds a counselor, finds a nonprofit, or uploads the bill to a stranger.

- Hospital counselors and screening vendors (Experian): only the minority who ask get screened; the patient never sees the math
- Dollar For: free, volunteers help you apply, national; generic process, limited capacity, no instant answer, not Maryland's tiers
- AI bill checkers (Bill Bodyguard, MedBillAI, BillAudit AI): your bill goes to their servers; they hunt errors, not eligibility
- CareRoute Bill Defense (2026): negotiates Maryland bills for a share of savings on a bill the law often says should be $0
- freecare: Maryland's exact tiers, in the browser, files the state form. Free for patients, paid for by hospitals

BUSINESS MODEL

Free for patients. Hospitals pay for the front door the new rules require.

- Patients: $0, always. No fees, no share of savings, no account, no upload
- Hospitals and health systems: a white-labeled license for the eligibility check and application on the bill and in the portal. Proposed $20 per completed application or an annual site license, a hypothesis to test in five hospital conversations
- Why they pay: eligibility must be determined before collection; Maryland rates already fund predicted uncompensated care, so turning bad debt into assistance costs little (at most 0.7% of net revenue nationally, Dollar For); fewer collection fees and lawsuits

GROWTH AND USE OF FUNDS

Three months: from a checker to a filing agent with real Maryland patients.

- November: public URL in English and Spanish; 20 patient and 5 counselor interviews; first clinic or legal-aid partner
- December: bill photo reader; policy picker for every HSCRC hospital; 100 families screened
- January to February: uniform application autofill; first hospital pilot conversation; pick the second state
- $4,500 award: health-law review and LLC $1,500; model costs, hosting, domain $1,000; Spanish version $800; clinic outreach in Prince George's County $700; reserve $500

TEAM

Built by someone who has watched his family stare at a hospital bill.

- Ayush Kadigari, founder. UMD '27, Information Science and Economics
- Ships privacy-first browser tools: Zenkei, Clause, Meridian. Investment banking intern at Watercress Capital; business analyst at GoGuardian; TAMID analyst
- Shipped freecare v1 in September 2026: Maryland's full tier staircase, 53 automated tests, zero network requests
- Looking for a co-founder from UMD Public Health or Law, and a hospital financial counselor as an advisor